CHAPTER 10
Child and Adolescent Mental Health

Understanding development, recognizing distress, building supportive families and schools, responding to bullying, and finding appropriate care.

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For caregivers, young people, educators, community workers, and readers seeking accurate introductory information. Not a diagnosis or a substitute for clinical assessment.

Safety and safeguarding: If a child is in immediate danger, seriously injured, or at imminent risk of self-harm, seek urgent local emergency help. In Rwanda, the government lists 112 for emergency help, 912 for ambulance, and 116 for the child helpline. Verify current availability and follow local child-protection procedures. [R5]

Learning goals and how to use this chapter

- Understand how mental wellbeing develops from childhood through adolescence.
- Recognize broad groups of mental health challenges without using symptom lists as diagnoses.
- Identify school, family, social and online pressures alongside protective relationships.
- Practice safe, age-appropriate conversation and coping skills.
- Know when routine support is insufficient and how to seek professional or urgent help.

This chapter uses fictional Rwanda-relevant scenarios. They are illustrations, not reports of real children. Caregivers and educators should adapt activities to a child's developmental level, disability, language, privacy needs and preferences. Avoid forcing disclosures.

1. What does child and adolescent mental health mean?

Child mental health includes emotional wellbeing, developing skills to manage feelings, building friendships, learning, and participating in family and school life. Having good mental health does not mean a child is calm or happy every day. Children may feel frustrated, frightened, angry, or sad while still growing in healthy ways. [R1, R2]

Adolescence commonly refers to ages 10–19. It brings rapid physical, cognitive and social changes. Young people increasingly seek autonomy, explore identity, become sensitive to peer relationships and manage growing responsibilities. These changes are important, but they do not always happen at the same speed. [R1]

The World Health Organization estimated in 2025 that approximately one in seven adolescents aged 10–19 lives with a mental health condition worldwide. This is a global estimate—not a Rwanda-specific prevalence figure—and shows why accessible, non-stigmatizing support matters. [R1]

2. Emotional development across life stages

Younger children often communicate distress through behaviour, play, sleep, appetite or physical complaints before they can describe complex feelings. They need adults who help name emotions, keep predictable routines and model calm responses. This does not mean every tantrum or fear is a mental illness.

School-age children usually develop more complex friendships and a growing sense of fairness, achievement and belonging. Academic pressure, conflicts, exclusion, family changes and difficulties with learning can affect mood and confidence. Adults can support them by listening and responding with consistent, fair boundaries.

Adolescents may want privacy, independence and greater control over decisions. Respectful family relationships balance this growing autonomy with appropriate safeguarding. A change in interests or desire for time alone can be normal; a persistent decline in functioning or safety

requires attention. [R1, R2]

Life stage	Developmental needs	Helpful adult response
Early childhood	Co-regulation, stable relationships, play	Name feelings; create safety and routines
School-age	Peer belonging, learning, mastery	Notice school pressures; encourage skills
Adolescence	Autonomy, identity, connectedness	Listen privately; negotiate safe boundaries

3. Risk and protective factors

A child’s mental health is shaped by interacting biological, psychological and social factors. Family instability, discrimination, food insecurity, chronic illness, harmful punishment, abuse, bullying, isolation or major loss can increase risk. These experiences do not guarantee a disorder, and no single factor explains an individual child’s difficulties. [R1, R3]

Protective factors include caring, dependable adults; safe learning environments; accessible healthcare; friends and community support; opportunities for play and activity; respectful discipline; and skills such as problem-solving and emotional regulation. Protection is a shared responsibility, not a task for the child alone. [R3, R4]

4. Common mental health challenges

Anxiety and fear: Children may experience persistent worries, fear of separation, social situations or particular events. When fear is intense, prolonged, and interferes with attendance, sleep, relationships or daily activities, assessment may help.

Depression and low mood: Signs may include sustained sadness or irritability, reduced interest, hopelessness, withdrawal or changes in energy and sleep. Irritability in adolescents can be prominent; it should not simply be dismissed as “bad behaviour.”

Attention, behaviour and learning: Persistent attention difficulties, impulsivity, aggression, or school struggles can have different explanations, including neurodevelopmental needs, learning barriers, stress, trauma or sleep problems. Effective responses begin with careful assessment, not punishment.

Trauma, eating concerns and substance use: Some young people experience distress after frightening events, changes in eating or body image, or harmful use of substances. Each deserves sensitive, age-appropriate support and clinical evaluation when needed. [R1, R2]

5. What is a warning sign rather than a normal phase?

A single bad day, argument or lower grade usually does not establish a mental health condition. Pay attention to duration, intensity, change from the child’s usual behaviour, distress and impact on daily life. Persistent school refusal, marked withdrawal, severe sleep changes, repeated unexplained physical complaints, loss of enjoyable activities or significant impairment merit a supportive conversation and possible assessment.

Watch urgently for expressed wishes to die, suicidal behaviour, self-injury, extreme confusion, psychotic symptoms, immediate danger, severe inability to care for basic needs, or signs of abuse. Never wait for a routine appointment if danger appears imminent. [R1, R2]

> Do not label or diagnose: “This child has depression” or “This teenager has ADHD” should not follow from a web checklist. Document observations without judgement and consult an appropriately qualified clinician.

6. School experience and mental wellbeing

School can be a powerful place of belonging, but also a source of distress. Examinations, performance expectations, classroom humiliation, learning differences, peer exclusion and disciplinary practices can influence emotions and participation. Educators can improve wellbeing by building predictable routines, demonstrating fairness, welcoming differences and providing ways to ask for help.

When a learner struggles, private and respectful inquiry is preferable to public embarrassment. Teachers should record observable changes, involve safeguarding or student-support staff according to school procedures, work with caregivers where safe and appropriate, and help families access professional assessment when indicated. [R4]

7. Bullying, exclusion and cyberbullying

Bullying generally involves repeated aggression and a power imbalance; it may involve physical acts, insults, humiliation, exclusion, threats or online abuse. Even a single serious incident can require protection. Adults should take reports seriously, make a proportionate safety plan, avoid blaming the affected child, and investigate according to safeguarding procedures.

Online spaces may expose young people to harassment, unwanted sharing of private information, image-based abuse or coercion. Do not require a child to confront an aggressor. Before blocking accounts, collecting screenshots or altering devices, consider whether a harmful person monitors the child’s device and whether a safer adult or professional should assist. [R1, R4]

8. Supportive parenting and caregiving

Children benefit from warmth and consistent boundaries. Caregivers can listen first, acknowledge feelings, explain expectations without threats, offer manageable choices and repair relationships after conflict. “You seem quieter recently; would you like to tell me what has been difficult?” is more inviting than an accusation.

Caregivers should avoid calling children weak, lazy or attention-seeking because of emotional difficulties. Encourage regular sleep opportunities, balanced meals where possible, play, movement, trusted friendships and reasonable schedules without making lifestyle changes the only response to a serious problem.

If a parent or caregiver is the source of harm, the child needs an alternative safe adult and the appropriate safeguarding route—not forced family reconciliation. [R3, R4]

9. Communication that helps young people speak

Choose a private, calm time. Use open questions, listen without interrupting and reflect what you hear. The goal is not to extract a complete story immediately. Ask what feels safest or most helpful right now. A child can decline to answer an optional educational question, and a teenager’s privacy deserves respect within child-protection responsibilities.

Adults should explain confidentiality honestly: they will respect privacy as much as possible, but may need to involve safeguarding or emergency services if someone is in danger or law requires action. Do not promise to keep abuse or immediate danger secret. [R4, R6]

10. Helping a young person experiencing distress

First check immediate safety and basic needs. Offer calm company and validate feelings without assuming a diagnosis. Ask whether they would prefer a different trusted adult, school staff member or health professional. Make a small plan together: who to contact, a suitable time and place, and how to manage school or home demands while support is arranged.

When a child discloses violence or exploitation, do not question them repeatedly, demand evidence, suggest blame or confront the suspected person. Follow child safeguarding procedures and involve trained professionals. The child’s safety and developmental needs take priority. [R4, R6]

11. Mental health in digitally connected lives

Digital experiences can support friendship, learning and peer connection, but may also involve social comparison, disrupted sleep, unwanted contact, harassment or exposure to distressing content. Ask what the young person experiences online instead of assuming all screen time has the same effects.

Agree on age-appropriate privacy, bedtime and safety practices. Adults should model respectful online behaviour and avoid secretive surveillance when a collaborative, safer approach is possible. If there is a credible threat, exploitation or urgent safety risk, safeguarding needs may override ordinary privacy expectations.

12. Supporting diverse needs and inclusion

Children with disabilities, neurodevelopmental differences, chronic illnesses, learning difficulties, displacement histories or experiences of discrimination may face additional barriers to care. Avoid assuming that all distress reflects an existing diagnosis or disability.

Adapt communication by offering simple language, visual supports, extra response time, private alternatives to public sharing, and an accessible route to care. Ask families and young people what accommodations work for them. [R1, R2]

13. Evidence-based care and professional assessment

A clinician may speak with the child, caregivers and relevant educators, with appropriate privacy and consent procedures, to understand symptoms, context, functioning, medical history and safety concerns. Care may involve psychological interventions, family support, school adjustments, targeted services or medication when clinically indicated.

WHO recommends evidence-informed psychosocial interventions that build social-emotional skills and prevent mental health problems among adolescents. These are not substitutes for clinical treatment when a condition requires it. Do not recommend medicines, stop existing treatment or conduct trauma-processing therapies through self-help content. [R3]

14. When and how to seek help in Rwanda

For non-emergency concerns, start with a qualified health professional, appropriate local health centre or hospital, and ask about child or adolescent mental health referral options. The Ministry of Health’s Kigali Mental Health Referral Centre publicly lists child and adolescent services; its published contact numbers are +250 793 902 059 and +250 736 440 666. Check current hours, service eligibility and referral arrangements before relying on them. [R7]

For immediate danger or imminent self-harm risk, seek urgent medical or emergency assistance. Rwanda’s government emergency directory lists 112 for emergency assistance, 912 for ambulance and 116 for its child helpline. These are published numbers, not a guarantee that each line is staffed or suitable for every case at every moment. [R5]

Five Rwanda-relevant fictional learning scenarios

A. Examination pressure

Situation: A 15-year-old in secondary school has trouble sleeping before national examinations, avoids friends and fears disappointing family.

Supportive response: A trusted teacher listens privately and checks functioning and safety. Caregivers reduce shaming and help organize realistic preparation and support. If distress persists or worsens, arrange professional assessment.

B. Younger child withdrawing

Situation: A 9-year-old stops joining games after a family move and becomes unusually tearful at school.

Supportive response: Adults ask gently about the transition, restore predictable routines, help build connections and monitor whether symptoms persist or impair daily life.

C. Bullying and school avoidance

Situation: A 13-year-old repeatedly avoids school after peers mock their speech in class and online.

Supportive response: School staff address the bullying under safeguarding procedures; the child is not required to confront peers. Adults plan safety and supportive educational adjustments.

D. Online distress

Situation: A 16-year-old has become anxious after receiving threatening messages through a shared phone.

Supportive response: A trusted adult checks whether the phone is monitored, considers safe documentation and reporting, and helps the teenager access appropriate support without blaming them.

E. Caregiver concern about mood

Situation: A caregiver notices their 17-year-old has lost interest in football and frequently says they feel hopeless.

Supportive response: The caregiver listens, asks directly and calmly about safety, and arranges professional assessment promptly. If there is immediate danger, seek urgent help.

Five practical learning activities

1. Feelings vocabulary (children)

How to practice: Invite the child to choose words or drawings describing a feeling from today. Ask what helped and whether they want to share more. Do not score feelings as right or wrong.

Safety note: Optional; no forced disclosure or public sharing.

2. Circle of trusted support (ages 8+)

How to practice: On paper, write or draw up to three people or services the learner could approach if worried. Include a backup if the first person is unavailable.

Safety note: Do not list a person who may be unsafe; respect a child who cannot name anyone and help identify safe services.

3. Stress-to-action map (adolescents)

How to practice: Make two columns: “What is worrying me?” and “One safe, manageable next action.” Choose only one low-risk next step.

Safety note: Do not use this exercise to solve situations of abuse through self-reliance.

4. Caregiver listening practice

How to practice: One adult rehearses a supportive opening sentence, one open-ended question, and one nonjudgmental reflection. Then switch roles.

Safety note: Do not dramatize an actual child’s confidential disclosure.

5. School inclusion checklist

How to practice: Educators review one classroom practice: participation options, respectful correction, anti-bullying responses, and referral routes. Identify a realistic improvement.

Safety note: Follow school safeguarding policies; record no identifying child details in training materials.

Knowledge check: questions and explained answers

1. Does every emotional change in adolescence indicate a disorder?

Answer: No. Consider persistence, severity, changes from usual behaviour and effects on everyday functioning.

2. What is a protective factor?

Answer: A condition that supports wellbeing, such as a dependable adult, safer school or accessible care.

3. Should a teacher publicly challenge a distressed learner to explain?

Answer: No. Provide a private, respectful opportunity and follow safeguarding procedures.

4. Can bullying affect school attendance and mental health?

Answer: Yes. Safety and support matter, and repeated power-imbalanced aggression requires attention.

5. Is a symptom checklist enough to diagnose ADHD or depression?

Answer: No. Assessment considers context, history, functioning and alternative explanations.

6. What should adults say about confidentiality?

Answer: Explain privacy respectfully and clarify safeguarding and safety-related limits.

7. Should survivors be forced to repeat traumatic disclosures?

Answer: No. Listen calmly, avoid unnecessary questioning and connect to trained support.

8. Can all sleep and mood difficulties be managed only with routines?

Answer: No. Persistent or severe problems may require professional assessment and treatment.

9. What makes digital-safety decisions complicated?

Answer: Monitoring by a harmful person can make blocking or changing settings risky.

10. What happens if there is immediate risk of self-harm?

Answer: Seek urgent local emergency or medical support; do not rely on an ordinary appointment.

Short glossary

Mental wellbeing: The emotional, psychological and social functioning that supports daily life.

Co-regulation: An adult helping a child manage overwhelming emotions through calm, attuned support.

Protective factor: A condition or resource associated with safer development and resilience.

Bullying: Repeated aggression involving a power imbalance.

Safeguarding: Actions to protect children from harm and respond appropriately to safety concerns.

Psychosocial intervention: Structured support addressing thoughts, behaviour, relationships or social circumstances.

Referral: Connecting someone to a service qualified to assess or provide needed support.

References and evidence register

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Editorial and publication safeguards

This chapter is an original, educational synthesis of the references above. It is not yet approved for public distribution. Before publication, obtain review from a qualified child/adolescent mental health professional and child safeguarding specialist; verify Rwanda-specific contact details and legal requirements; validate any Kinyarwanda or French translation; and ensure accessible formatting. Review clinical guidance and referral information periodically.